

PETITION TO THE INDIANA GENERAL ASSEMBLY

For a state direct care staffing floor, protection of the caregivers who report, and accountability for nursing home Medicaid dollars

To the Honorable Speaker and Members of the House of Representatives, and the Honorable President Pro Tempore and Members of the Senate, of the 125th General Assembly of the State of Indiana:

The undersigned, residents of the State of Indiana, exercising the right secured by Article 1, Section 31 of the Constitution of the State of Indiana to instruct our representatives and to apply to the General Assembly for redress of grievances, respectfully represent:

WHEREAS, Indiana law sets no minimum number of certified nurse aide hours or total direct care hours per resident day in comprehensive care facilities, requiring only one-half hour of licensed nurse time per resident day under 410 IAC 16.2-3.1-17, and Indiana is one of eighteen states with no direct care staffing minimum of any kind;

WHEREAS, an analysis of federal payroll-based staffing data published March 18, 2025 ranked Indiana fiftieth of the fifty states and the District of Columbia for total nurse staffing hours per resident after adjusting for resident acuity, and federal data for the first quarter of 2026 show that eighty-eight percent of Indiana nursing homes staffed below their residents' assessed needs;

WHEREAS, the federal minimum staffing standard finalized in May 2024 was repealed effective February 2, 2026, and Public Law 119-21 bars its enforcement until after September 30, 2034, so that no federal floor will exist for the remainder of this decade unless the State of Indiana sets one;

WHEREAS, county-owned hospitals hold the licenses to most Indiana nursing homes and, according to published investigative reporting drawing on records obtained through litigation, retained roughly \$2.6 billion of \$5.6 billion in supplemental Medicaid payments generated by those homes over approximately fifteen years, applying it to hospital construction, equipment, and operations rather than bedside care;

WHEREAS, a resident's rights are exercised through the frontline caregiver, and Indiana's at-will employment law, together with a thirty-day window to file a retaliation complaint, leaves that caregiver exposed to discipline, termination, and nurse aide registry referral for acts and omissions caused by conditions the facility created; and

WHEREAS, the safety of residents and the safety of the workers who care for them are one problem, and no resident right can be enforced by a worker who cannot act on it without losing their livelihood;

NOW, THEREFORE, your petitioners respectfully pray that the General Assembly:

1. Introduce, hear in committee, and enact in the 2027 regular session the **Indiana Nursing Home Direct Care Staffing and Accountability Act**, adding IC 16-28-17, IC 16-28-18, IC 16-28-19, IC 16-28-20, and IC 16-22-15 to the Indiana Code and amending IC 16-28-5-4 and IC 16-28-9-3;
2. Establish thereby a minimum direct care staffing standard of 3.00 total hours per resident day beginning July 1, 2027 and 3.48 hours beginning July 1, 2028, including 2.45 certified nurse aide hours and a

registered nurse on site twenty-four hours a day, with acuity-based staffing above the minimum, annual unannounced acuity verification surveys, and public posting of staffing on every shift;

3. Protect every direct care employee who reports an unsafe condition or refuses an unsafe assignment, through a rebuttable presumption of retaliation, a private right of action, and a systemic cause determination before any nurse aide registry finding, discipline, or termination for an act or omission caused by staffing below the standard, missing or inoperable equipment, an inaccurate assist level, or facility direction;
4. Provide that penalties for staffing and retaliation violations exceed the cost of compliance, are not reduced upon correction, are not paid from Medicaid funds or charged to residents, and are deposited in a direct care restitution fund;
5. Require county hospitals that hold nursing facility licenses to spend not less than eighty percent of the nursing facility supplemental Medicaid payments attributable to a facility on direct care at that facility, to file annual public facility-level accountings, and to repay diverted amounts; and
6. Recognize the direct care workforce as a party to nursing home policy in Indiana, by consulting the Care Floor Coalition and the facility direct care staffing committees created by the Act in the development of rules, reimbursement, and oversight affecting long-term care, and by inviting direct care workers, residents, and families to testify at any hearing on the Act.

Respectfully submitted by the undersigned residents of Indiana, whose names appear on the attached paper signature sheets and on the Schedule of Signatures Collected Online (Exhibit B), each of whom has signed this petition once on the date shown.

Presented to the [House of Representatives / Senate] by the Honorable _____,
District _____, on _____, 20____, for referral to the Committee on
_____.

Submitted on behalf of the petitioners by The Care Floor Coalition · dezzerea70@gmail.com. Total
signatures attached: _____ on _____ sheets.

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Signature Sheet 1 of 3 · attach to the petition text

We, the undersigned residents of the State of Indiana, exercising the right secured by Article 1, Section 31 of the Constitution of the State of Indiana, join in the Petition to the Indiana General Assembly set out on the attached pages, and respectfully pray that the General Assembly enact the Indiana Nursing Home Direct Care Staffing and Accountability Act in its 2027 regular session, protect the direct care workers who report unsafe conditions, require nursing facility supplemental Medicaid payments to be spent on direct care, and recognize the direct care workforce, through the Care Floor Coalition, as a party to nursing home policy in Indiana. Each of us has signed once, in our own handwriting, on the date shown.

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Signers must be residents of Indiana. Complete your own line in your own handwriting; a signer with a disability may authorize another person to write for them. Use your residence address, not a work address. One signature per person across all sheets. Nothing here is published; the petition is delivered to your own state representative and senator.

Circulator's statement. I circulated this sheet and, to the best of my knowledge, each signature was made in my presence by the person whose name appears beside it.

Circulator's signature _____ Printed name _____

County _____ Date _____ Return to: The Care Floor Coalition ·
dezzerea70@gmail.com

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